Dashboard Disclaimer & Limitations
Maintained by: Paty Rivera Norton
Last updated: April 2026

This dashboard is designed to support clinical decision-making by surfacing trends, flags, and productivity metrics from data recorded in Exym. It is intended as a **decision-support tool only**. Dashboard outputs should always be interpreted in context and verified in Exym before any clinical, administrative, or billing action is taken.

Data Refresh & Lag

- Dashboard data reflects Exym activity with an approximate **24-hour delay**. Activities entered, edited, or completed today will generally not appear until the following day.
- This means that **same-day sessions, note submissions, and status changes are not visible in real time.** A clinician who completed a session this morning will not see it reflected in their MTD hours or progress metrics until tomorrow.
- If a metric looks unexpectedly low or a flag appears to conflict with recent activity, check Exym directly before drawing conclusions or taking actions.

Medi-Cal Eligibility

- The eligibility data powering the **"No Medi-Cal"** flag comes from a manually updated monthly table. It is **not a live eligibility check** and reflects a point-in-time batch verification, typically run once per month.
- A client flagged as ineligible may have had their eligibility reinstated since the last table update. If a client tells you that an eligibility determination change has occurred, please contact Paty, data team, intake or front desk to confirm.

Productivity Goals & Pacing

Monthly Hour Goals Are Static

- The dashboard applies a fixed monthly goal of **80 hours for clinicians** and **40 hours for interns**. These figures do not dynamically adjust for:

- **Clinicians who recently joined** mid-month or mid-fiscal year. A new hire in week three of the month is held to the same 80-hour standard as someone who has been with the program all month. Pacing and projected metrics will appear behind accordingly.
- **Planned time off, medical leave, or reduced schedules.** The dashboard has no visibility into approved absences and will flag pace deficits even when the clinician is on approved leave.
- **Supervisors or leads** who carry a reduced direct service expectation due to administrative responsibilities. Their dashboard metrics should be interpreted in light of their individual expectations, not the standard goal.
- **Caseload transitions**- a clinician inheriting clients mid-month or transferring a large portion of their caseload will see distorted productivity and engagement metrics during the transition period.

Pacing Calculations Exclude Holidays

- The business day calculations used for pacing and projections account for weekends only. **Observed holidays are not automatically excluded.** A week with two holidays will still show the full business day count, making pace appear lower than it actually is relative to available working time.

Supervisor Allotment & Team Composition

- The supervisor dashboard reflects a team built from two sources: clinicians assigned under a supervisor's ID in Exym, and a secondary mapping table that accounts for edge cases (supervisors overseeing staff not formally linked in the system).
- This mapping is maintained manually and may not immediately reflect **reassignments, departures, or new supervisory relationships** that haven't been updated in both places.
- A clinician who appears on a supervisor's dashboard may have recently been reassigned, and one who is absent may not yet have been added. If the team roster looks incorrect, contact Paty Rivera Norton to review the mapping.
- **Staff activity proxy:** Because Exym's termination and hire status fields are not always updated in real time, the dashboard uses a rolling 60-day window of billable activity to determine whether a staff member is considered active. Someone on

extended leave or who recently separated may still appear on the team roster if they had activity within that window.

Alert Flags

The flags generated by this dashboard, including service gaps, high no-show rates, late documentation alerts, and client risk indicators, are **rule-based approximations**, not empirical assessments. They are put in place to relay patterns that may warrant attention, not to replace clinical judgment.

Specific limitations to be aware of:

- **Service gap flag (30 days):** Triggers when no billable activity appears in the last 30 days. It does not account for planned treatment breaks, hospitalizations, or other valid reasons for a gap.
- **No-show rate:** Calculated from the no-show records in Exym for the current month. If a no-show was entered in error or later corrected to a kept appointment, the correction may not reflect immediately.
- **Late documentation flag:** Based on business days elapsed between the session date and the first note submission. It does not account for extenuating circumstances, training periods, or supervisor-approved extensions.
- **Care plan expiration:** Flags plans expiring within 30 days. The underlying data depends on care plan end dates being accurately entered in Exym.
- **Turning 18 / Turning 21:** Based on date of birth in Exym. If DOB is entered incorrectly, the flag may be inaccurate.
- **Month streak (engagement):** Counts consecutive months with at least one billable session. A streak break does not necessarily indicate disengagement. It may reflect a month of approved leave, hospitalization, or other circumstances.

What This Dashboard Cannot Do

- It cannot replace a clinical conversation between a supervisor and intern/clinician.
- It cannot determine medical necessity, clinical appropriateness, or billing eligibility.
- It cannot account for documentation or activity entered in a system other than Exym.

- It cannot reflect real-time changes — all data is historical as of the prior day's database sync.

Questions & Corrections

If you believe a metric is incorrect, a client is missing, or a flag is being triggered in error, please contact:

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This document is reviewed and updated periodically. Clinicians and supervisors are encouraged to use dashboard data as one input among many, alongside direct communication, Exym records, and their own professional judgment.